



Psoriasis and Papulosquamous Diseases

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IS-NMSPD- Fall, 2025

Special thanks to Dr. Pfothenhauer and Dr. St. Pierre. Lecture was built upon, adapted and updated from their prior lecture.

Learning Objectives

◆ For EACH of the following disorders, know the objectives A-E

1. Psoriasis vulgaris and variants
2. Pityriasis rubra pilaris
3. Pityriasis rosea
4. Lichen Planus

◆ Objectives A-E

- A. Identify the key epidemiologic features
- B. List the etiology, triggers, and pathophysiology
- C. Recognize the description or picture of a typical presentation
- D. Describe the most commonly affected areas
- E. Choose the most appropriate therapy



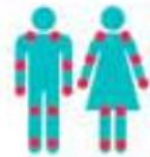
Psoriasis

Epidemiology

Affects 1-3% of the population worldwide

7.5 Million
Americans 125
million Worldwide

PSORIATIC ARTHRITIS™



30%
of patients are affected



75%
FEEL
UNATTRACTIVE



54%
FEEL
DEPRESSED

MOST COMMON SYMPTOMS™



THICK,
RED SKIN



SCALING



ITCHING



PAIN

**Typically
develops
between the
ages of 15-35**

Pathophysiology

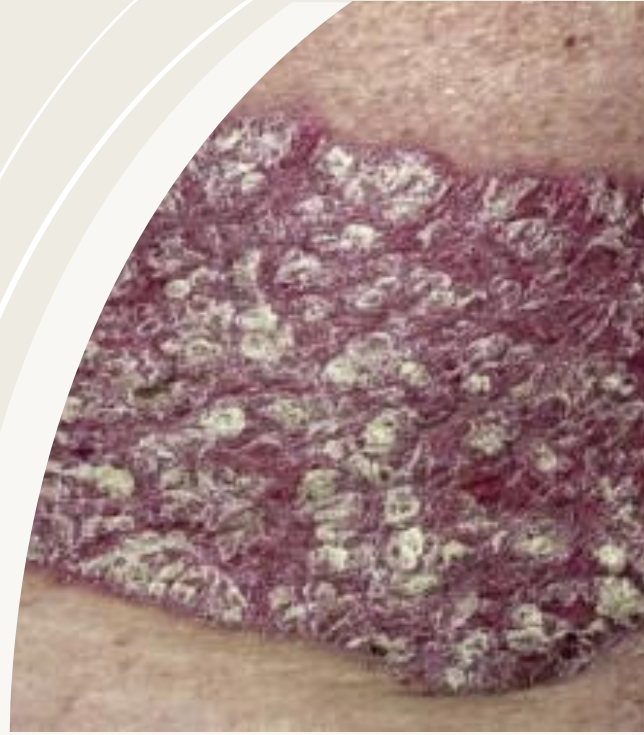
- ◆ Complex immune-mediated disease of T cells, that stimulates hyperproliferation of keratinocytes that create plaques

Risk Factors/Triggers

- ◆ Genetic Predisposition
- ◆ Physical trauma (Koebner phenomenon)
 - ◆ scratching, sunburn, or surgery
- ◆ Infection
 - ◆ Acute streptococcal infection precipitates guttate psoriasis
 - ◆ HIV
- ◆ Drugs
 - ◆ Beta blockers, oral lithium, antimalarial drugs, systemic glucocorticoids, TNF alpha inhibitors
- ◆ Smoking, Obesity, Alcohol use

Clinical Manifestations

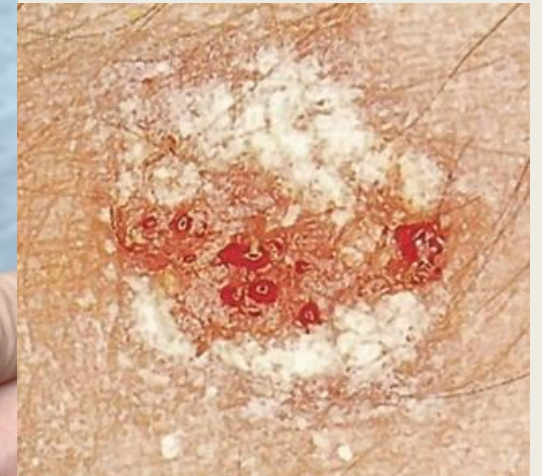
- ◆ *Salmon-colored, erythematous, sharply margined plaque, with thick silvery-white scales*



Clinical Manifestations

◆ Classic signs

- ◆ **Auspitz sign:** pinpoint bleeding after removal of scale overlaying psoriatic plaque
- ◆ **Koebner phenomenon:** development of skin disease in sites of skin trauma (note- this can also be seen in other skin diseases, ex: lichen planus and vitiligo)





(A) Silvery white annular plaques on the back, an appearance that can occur in patients with any skin type. **(B)** Localized plaque psoriasis affecting the elbow. Commonly seen in patients with Fitzpatrick skin types 1 through 3. **(C)** Violaceous, erythematous plaques on extensor surface of bilateral lower legs. Commonly seen in patients with Fitzpatrick skin types 4 through 6. **(D)** Plaque psoriasis appearing as bluish, annular plaques on extensor surface of bilateral lower legs. Commonly seen in patients with Fitzpatrick skin types 4 through 6.

***Fitzpatrick skin types:**
 - 1 through 3 (always, usually, or sometimes sunburns)
 - 4 through 6 (rarely sunburns, brown or black skin)

Commonly Affected Areas

- ◆ *(extensor surfaces)*
Elbows, knees, scalp, gluteal cleft
- ◆ Can occur anywhere but is typically found in these areas



Source: Klaus Wolff, Richard Allen Johnson, Arturo P. Saavedra.
Fitzpatrick's Color Atlas and Synopsis of Clinical Dermatology,
7th Edition, www.accessmedicine.com
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Psoriasis classification

- ◆ Psoriasis vulgaris (“Common” psoriasis)
 - ◆ Chronic stable plaque psoriasis
 - ◆ Acute guttate psoriasis
 - ◆ Pustular psoriasis
 - ◆ Erythrodermic psoriasis
 - ◆ Special sites
 - ◆ Scalp
 - ◆ Nail psoriasis
 - ◆ Inverse (intertriginous) psoriasis
 - ◆ Palmoplantar psoriasis

Chronic Stable Plaque Psoriasis

- ◆ **Most common presentation**
- ◆ Chronic, well-defined plaques.
- ◆ Irregular, round to oval, with predilection for extensor surfaces
- ◆ ***Symmetrically distributed***
- ◆ ***Involves nails (pitting, onycholysis)***
- ◆ May be pruritic
- ◆ Remain stable for months or years





Acute Guttate Psoriasis

- ◆ *Salmon-pink papule 2mm-1cm with or without scales ('dew-drop' lesions)*
 - ◆ Scales become evident on scraping
- ◆ Papules suddenly appear on trunk and extremities
 - ◆ Scalp and face may be involved but palms and soles are not
- ◆ More than 30% of pts have first episode before age 20 and is often an episode of guttate psoriasis



Acute Guttate Psoriasis

- ◆ ***Strep pharyngitis*** or viral URI may precede eruption
 - ◆ Throat cx should be taken and pts with strep infection should be treated with abx
- ◆ Guttate psoriasis ***may spontaneously remit***, typically over the course of several weeks to several months, may intermittently recur, or may persist and progress into chronic plaque psoriasis





Source: Klaus Wolff, Richard Allen Johnson, Arturo P. Saavedra:

Pustular Psoriasis

- ◆ Rare, abrupt onset, can be life-threatening
- ◆ Characterized by pustules, not papules, arising on normal or inflamed, erythematous skin
- ◆ Location: generalized, or localized to hands and feet
- ◆ Pustules coalesce to form “lakes” of pus
 - ◆ Easily wiped off
- ◆ Fever, malaise, leukocytosis
- ◆ Symptoms: burning, painful, pt appears frightened

Pustular Psoriasis

- ◆ Triggers: medications, **systemic steroids** (one of the few autoimmune conditions where systemic steroids is not helpful), pregnancy, infections, stress
- ◆ Pts should be hospitalized for hydration, antibiotics to prevent infections / wound care, and topical and systemic treatments IV biologic (spesolimab)





Erythrodermic Psoriasis

- ◆ Rare, severe generalized disorder
- ◆ Deep erythema, exfoliation, and associated abnormalities of temperature and cardiovascular regulation
- ◆ Generalized distribution from head to toe
- ◆ Generally follows excessive UV exposure, systemic steroids, excessive use of topical steroids, or stress
- ◆ Severe cases may require hospitalization – associated with electrolyte derangements and systemic infections



Special Sites: Nails

- ◆ Fingernails and toenails frequently involved (25%), especially with concomitant arthritis
- ◆ *Can see pitting, subungual hyperkeratosis, onycholysis and yellowish-brown spots under the nail plate (called the **Oil Spot**)*



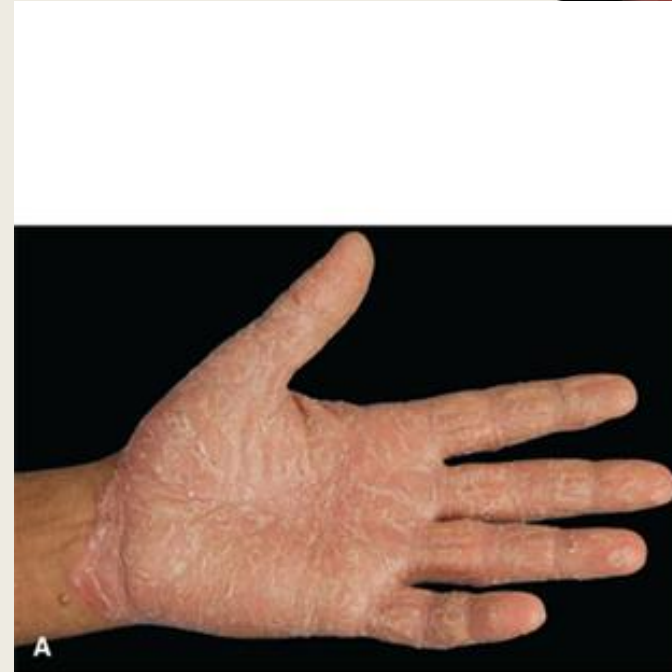
Special Sites: Nails

- ◆ Associated nail changes: *pits, oil spots, nail loss*



Special Sites: Palms and Soles

- ◆ Massive silvery white or yellowish hyperkeratosis and scaling
- ◆ Not easily removed
- ◆ Desquamation of hyperkeratosis reveals an inflammatory plaque at the base that is always sharply demarcated
- ◆ May be cracking, painful fissures and bleeding



Source: Klaus Wolff, Richard Allen Johnson, Arturo P. Saavedra:
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Special Sites: Palms and Soles

- ◆ Deep pustules in the center of the palms or instep of soles
- ◆ Low incidence
- ◆ Chronic relapsing eruption limited to palms and soles
- ◆ Exacerbations may be very painful and limit mobility
- ◆ Not an emergency



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Special Sites: Scalp

- ◆ Plaques sharply marginated with thick adherent scales
- ◆ Scattered discrete or diffuse involvement of entire scalp
- ◆ Often very pruritic



Scale may become extremely dense, especially on the scalp.



Special Sites: Inverse Psoriasis

- ◆ Plaques are often not scaly but are macerated, often bright red and fissured due to warm moist environments of these regions
- ◆ Located in flexural intertriginous creases, such as perianal ,genital regions, and of the body folds
- ◆ Sharp demarcations permits distinction from candidiasis, contact dermatitis, or tinea cruris



Special Sites: Inverse Psoriasis

In intertriginous areas, scale forms but is macerated and dispersed therefore appearing as smooth, red plaques. This is common in the intergluteal fold and referred to as gluteal pinking



Psoriatic Arthritis

- ◆ 30% of patients with psoriasis develop psoriatic arthritis
- ◆ Onset may occur at any age but peaks ages 20-40
- ◆ **Very debilitating**
- ◆ Chronic, progressive inflammatory arthropathy of the peripheral joints, spine/SI, and entheses
- ◆ Seronegative- rheumatoid factor and anti-CCP are negative
- ◆ +/- HLA B27
- ◆ May precede, accompany, or, more often, follow skin manifestations

Psoriatic Arthritis

- ◆ Asymmetric peripheral joint involvement of the upper extremities especially distal interphalangeal joints
- ◆ Dactylitis- Sausage fingers
- ◆ Nail involvement in 80% vs 30% without arthritis
- ◆ Enthesitis- inflammation at the site of ligamentous and tendinous insertion



Comorbidities Associated with Psoriasis

- ◆ **Autoimmune diseases:** Crohn's disease and ulcerative colitis are 3.8 to 7.5 times more likely
- ◆ **Cardiovascular disease:** more frequently overweight, increased incidence of DM, HTN, HLD
- ◆ Metabolic syndrome: impaired glucose, hypertriglyceridemia, decreased HDL, and HTN
- ◆ Vitamin D deficiency
- ◆ Depression/suicide: may be as high as 60%
- ◆ Emotional burden: increased rates of poor self-esteem, sexual dysfunction, and anxiety
- ◆ Alcohol: prevalence increased in those that abuse alcohol
- ◆ Obesity: higher BMI

Diagnosis of Psoriasis Vulgaris

- ◆ Diagnosis is **clinical**

- ◆ Differential includes:

 - ◆ Maculopapular drug eruption

 - ◆ Secondary syphilis

 - ◆ Pityriasis rosea

 - ◆ Tinea (capitis, corporis, pedis)

 - ◆ Seborrheic dermatitis

 - ◆ Onychomycosis

Psoriasis Treatment

- ◆ Except for erythrodermic / pustular psoriasis (requires prompt initiation of systemic drug therapy and inpatient management of fluids and electrolytes), all other types of psoriasis are treated similarly
- ◆ Stepwise approach
- ◆ **UV light**
- ◆ **Topical steroids**
- ◆ Others (methotrexate, calcineurin inhibitors, anti-TNF-alpha)

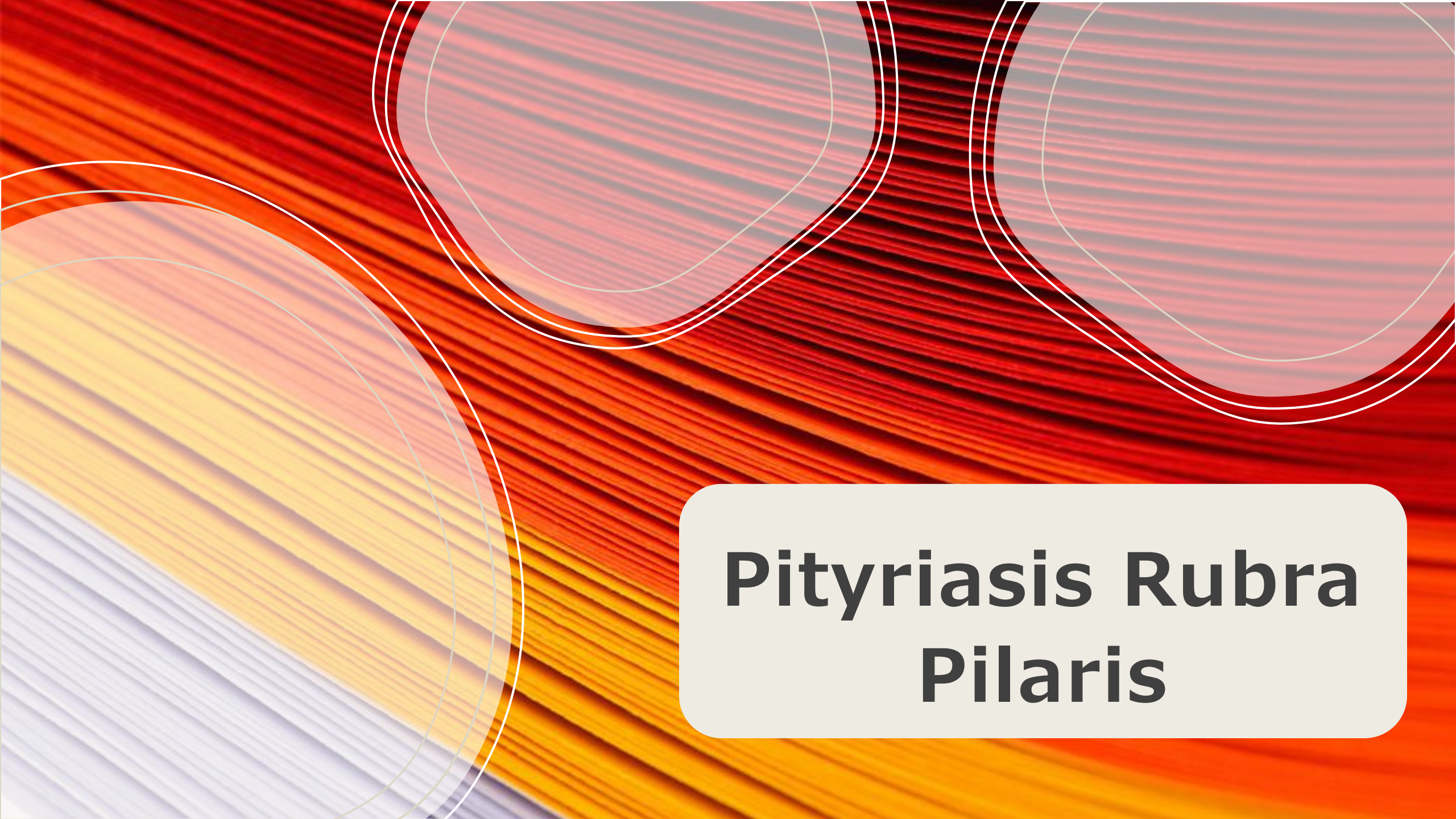
Psoriasis Treatment

Mild limited disease (topical agents only)

- ◆ Emollients
- ◆ Topical Vitamin D analogues (calcipotriene)
- ◆ Topical corticosteroids (clobetasol)
- ◆ Topical retinoids (tazarotene)

Moderate-Severe Disease (topical agents + systemic therapy)

- ◆ Phototherapy (ex: narrowband UVB, PUVA)
- ◆ Immunomodulators (ex: methotrexate, cyclosporin)
- ◆ Biologics (ex: anti-TNF alpha agents, anti-interleukin-17 agents)



Pityriasis Rubra Pilaris

Epidemiology, Etiology

- ◆ Frequency in U.S. is 1 in 3500-5000
- ◆ Affects both sexes, all races and can occur at any stage of life but most cases occur in the 1st or 5th-6th decade of life.
- ◆ Etiology is unknown



Pathogenesis

- ◆ Rare, chronic, papulosquamous disorder often progressing to erythroderma
- ◆ Pathogenesis is not fully understood, but interleukin 23/17 may play an important role due to response to therapies targeting IL-17 and IL-23
- ◆ Most cases are sporadic (acute onset)
 - ◆ Few cases are familial (gradual onset)
 - ◆ Acquired cases may be associated with HIV, strep, cancer



Clinical Presentation



- ◆ **reddish-orange, scaly plaques**, palmoplantar keratoderma and keratotic **follicular papules**
- ◆ Disease progresses cranio-caudally
 - ◆ Begins with small, indolent, red scaling plaques on the face or upper body that enlarge over days to weeks
 - ◆ **Palms and soles** begin to **thicken and bright red-orange follicular papule** appear on dorsal aspects of proximal phalanges, elbows, knees, and trunk as disease evolves into generalized eruption
 - ◆ **Islands of normal skin ("skip spots")**
 - ◆ Rarely may have mucosal involvement
- ◆ Diagnosis **is clinical**





Treatment

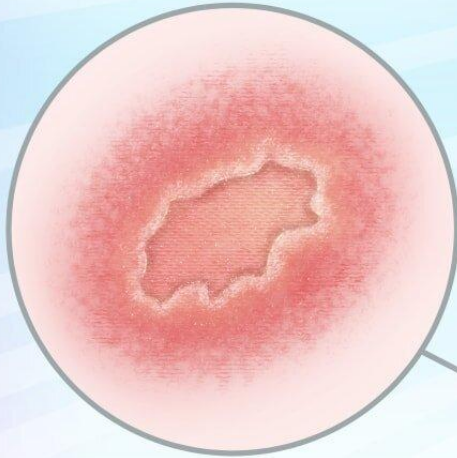
- ◆ There's no set treatment protocol but most patients will respond to **TNF-alpha inhibitors**
- ◆ Increased recognition of **IL 23/IL 17 axis** has contributed to increased use of biologic IL-17 and IL-23 inhibitors



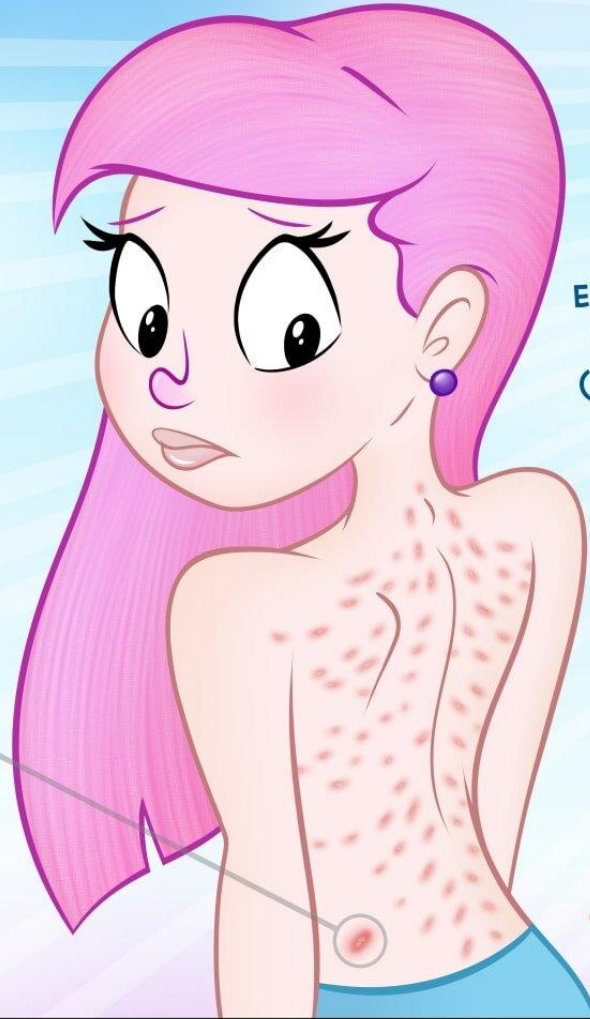
Pityriasis Rosea

PITYRIASIS ROSEA

AN ACUTE ERUPTION
OF OVAL, SCALY PAPULES AND
PLAQUES THAT BEGINS WITH
A SINGLE HERALD PATCH



1-2 WEEKS LATER THE ERUPTION
APPEARS ON THE TORSO IN A
CHRISTMAS TREE PATTERN



THIS IS
A **PITY.**

A VIRAL
ETIOLOGY IS
PROBABLE
(HHV-6 AND
HHV-7)

MAINLY SEEN IN
TEENAGE AND YOUNG
ADULT FEMALES



SUNLIGHT OR UVB
PHOTOTHERAPY MAY
HELP THE LESIONS
DISAPPEAR FASTER

CONDITION RESOLVES
SPONTANEOUSLY IN
ABOUT 6 WEEKS

Epidemiology, Etiology, Pathophysiology

- ◆ Acute, self-limited, benign, exanthematous skin disease
- ◆ >75% of pts are between the ages of 10-35
- ◆ Etiology: more study is needed, viral etiology has been hypothesized. Some evidence the human herpesvirus 6 and 7 (HHV-6 and 7) may be involved
- ◆ Incidence higher in the **colder months**
- ◆ Pathophysiology: cell activity suggest predominantly T-cell mediated immunity. May be associated with exanthem phase of viral infection, as patients may present with **URI before skin lesions in about 68% of pts**

Clinical Presentation

- ◆ 1st lesion: *herald patch*
 - ◆ *May be the only skin manifestation for ~2 weeks*
 - ◆ *Single 2-10cm round-to-oval lesion*
 - ◆ Most frequently seen on trunk or proximal extremities
 - ◆ May become scaly with central clearing
 - ◆ Often confused with ringworm



Clinical Presentation

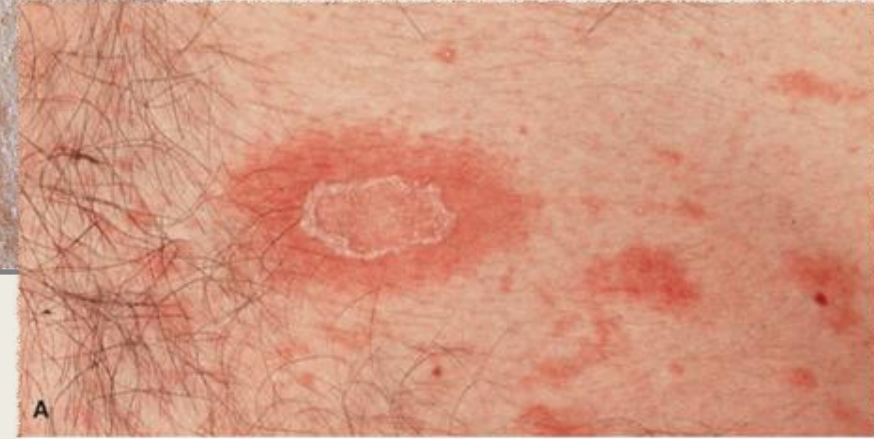
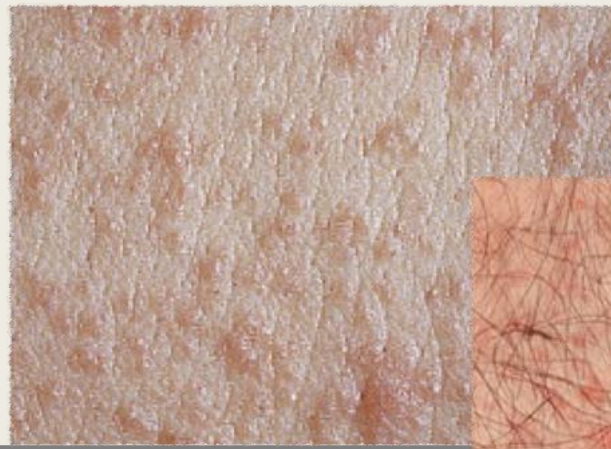
- ◆ A few days to 2 weeks after herald patch:
 - ◆ *Smaller lesions appear typically limited to trunk and proximal extremities*
 - ◆ Slightly inflammatory, oval, scaly, maculopapular lesions
 - ◆ On back tend to be oriented along the lines of cleavage of the skin/Langer Lines
 - ◆ ***Christmas Tree*** pattern on back
- ◆ Most lesions are asymptomatic to mild to severe itching





Diagnosis, Prognosis

- ◆ Diagnosis is **clinical**
- ◆ Most cases **self-resolve in 4-6 weeks, some take months**
- ◆ DDx: guttate psoriasis, secondary syphilis, tinea corporis, tinea versicolor, nummular eczema



Treatment

- ◆ Treatment: self-limited, so watchful waiting and reassurance.
- ◆ Can treat symptomatically
 - ◆ Ex: Topical steroids for itching





Lichen Planus

Epidemiology, Etiology, Pathophysiology

- Epidemiology: not well defined.
- Most frequently develops between 30-60 yo.
- Vulvar disease may be a common manifestation of lichen planus in women
- Etiology: unknown.
- Trigger: May be associated with *HCV in areas of high prevalence*, drug exposure
- Pathophysiology: T-cell mediated disease affecting the stratified squamous epithelia of skin and/or mucous membrane
- Most cases resolve in 1 year but recur in 49% of patients
- Buccal mucosal or genital involvement and pain can be severe and debilitating

Clinical Presentation



◆ Shiny, flat, polygonal, violaceous papules

◆ *The **P's** of lichen planus:*
***P**ruritic, **P**lanar (flat-topped), **P**olygonal, **P**urple **P**apules...
coalesces into **P**laques*



Clinical Presentation

- ◆ Typically affects the skin, nails, **mucous membranes (esp *mouth*)**, and the vulva and penis
- ◆ Flexor surfaces of the extremities (**wrists, forearms, legs**) are common sites
- ◆ ***Severe pruritus***. Lesions may become very painful if they ulcerate or erode
- ◆ Usually heals with significant post inflammatory hyper pigmentation



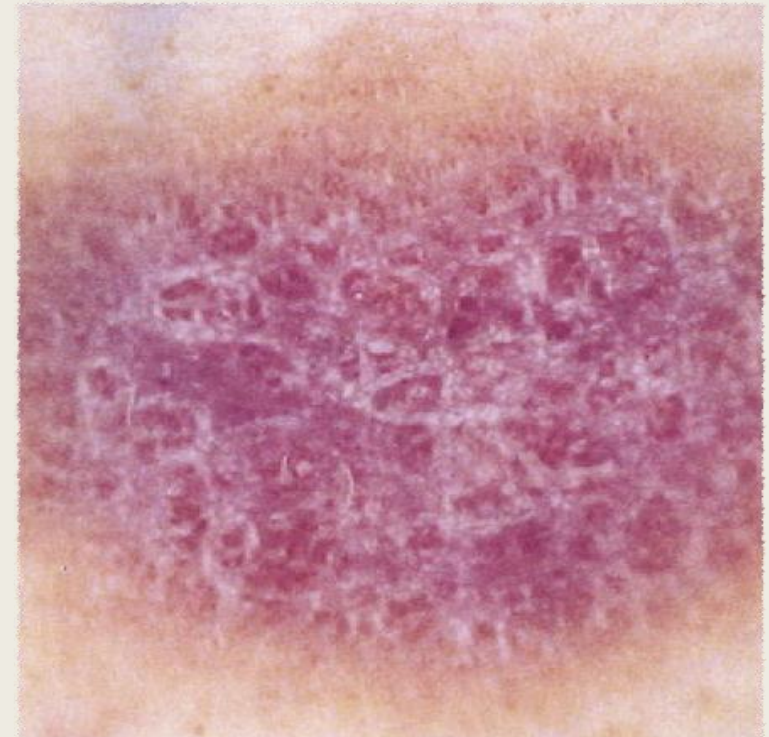


Clinical Presentation



◆ ***Wickham's Striae***

- ◆ Characteristic *white, lace-like patterns* on the surface of the papule and plaques
- ◆ May be accentuate by a drop of immersion oil



Clinical Presentation

- ◆ Koebner reaction
 - ◆ Development of skin lesions in areas of trauma such as scratching
- ◆ Nails may be involved from minor dystrophy to total nail loss



Other sites

◆ Oral

- ◆ Can include papular, atrophic, and painful erosive lesions



Other sites



◆ Genitals

- ◆ Men: violaceous papules on glans penis - annular
- ◆ Women: Vulvar Wickham's striae , erosions, scarring that results in adhesions. Painful desquamative vaginitis
- ◆ Ulcerative and resistant to treatment



Other sites

◆ Scalp

◆ Lichen planopilaris

◆ Erythematous follicular papules and extending smooth areas → can lead to scarring alopecia

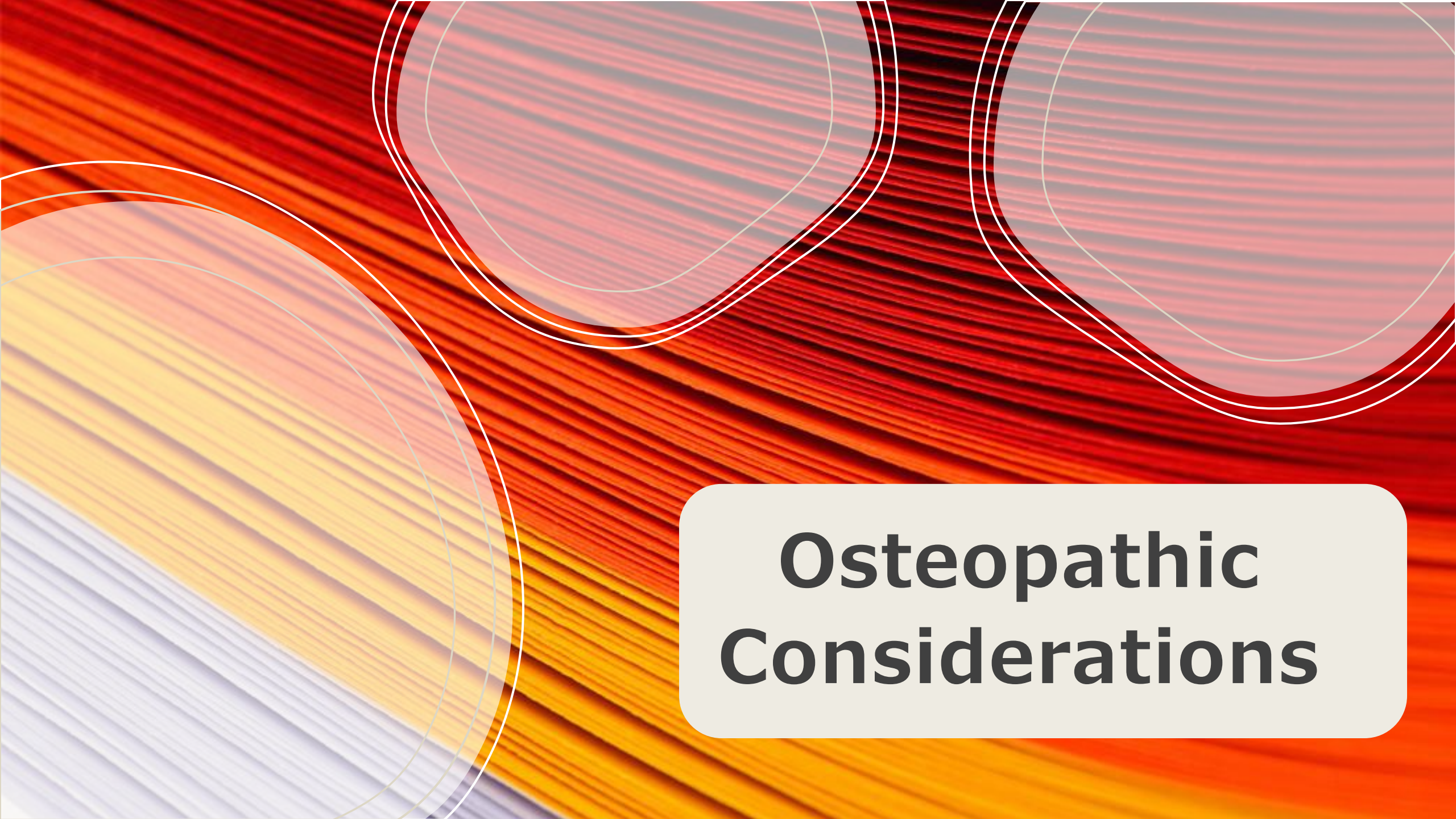


Diagnosis, Course

- ◆ Diagnosis **is clinical**
- ◆ DDx: chronic graft-vs-host, secondary syphilis, lymphoma, dermatomyositis
- ◆ Usually remits in 1-2 years
 - ◆ **lichen planus affecting mucous membranes may be more persistent and resistant to treatment**

Treatment

- ◆ Potent topical steroids – 1st line
- ◆ Intralesional steroids – 1st line
- ◆ Systemic glucocorticoids
- ◆ Oral retinoids
- ◆ Phototherapy



Osteopathic Considerations

Osteopathic Considerations

- ◆ Autonomics – optimize parasympathetics with treatment of OA and sacrum. Rib raising to balance sympathetics and help with lymphatic flow
- ◆ Biomechanics – treat somatic dysfunction near site of lesions. Extremities, spine, SI for psoriatic arthritis.
- ◆ Circulation – myofascial release to diaphragms (thoracic inlet, abdomen, pelvic) to help lymphatic flow
- ◆ Screening – Zink screen, diet, movement/exercise, environment, triggers, behavioral health support, social support
- ◆ Be mindful with open wounds



Cases

Case 1

A 25 year old male presents to your office with a 4 month history of “dry skin”. He states he has several patches of red, scaly skin on his elbows and lower back. He has tried lotions but it does not seem to make it better.

On exam you see erythematous, scaly patches with a silver scale on bilateral dorsal surface of elbows and lower back. Patches are sharply margined. When you scratch the scale, the skin begins to bleed.

Which of the following is the most likely diagnosis?

- ◆ 1. Psoriasis vulgaris
- ◆ 2. Pityriasis rubra pilaris
- ◆ 3. Pityriasis rosea
- ◆ 4. Lichen Planus

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Case 2

A 23 year old female presents to your office with a “rash”. She states she was diagnosed with strep throat about 1 week ago and given antibiotics. She thinks she had an allergic reaction to the medication because she now has red spots all over her stomach and back. She denies itching but has tried diphenhydramine which did not help.

On exam, you see small, pink, discrete papules and patches on her anterior and posterior trunk.

Which of the following is true about her condition?

- ◆1. Oral steroids are the best treatment
- ◆2. The rash can resolve spontaneously
- ◆3. She can only get this once
- ◆4. She should never take antibiotics again

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Case 3

An 18 year old male presents to your office with a rash. He just started college and is living in the dorms. He is afraid he may have bed bugs because his roommate had a similar rash. He denies itching or pain. He states it started with a large patch on his abdomen then spread to his chest and back.

On exam, you see erythematous, discrete, red, scattered papules on his abdomen and back. On his back, the pattern looks like a Christmas tree. There is a larger patch on his right lower abdomen that is approximately 3 cm x 5 cm oval, discrete, and erythematous.

Which of the following is true about his condition?

- ◆ 1. He will have this rash forever
- ◆ 2. Treatment is systemic steroids
- ◆ 3. This is a self-limited disease
- ◆ 4. He needs to treat his clothes and dispose of his mattress

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Case 4

A 40 year old female presents to your office with a rash. She states the rash began about 3 weeks ago. It is on her inner wrist and is extremely itchy. She tried diphenhydramine and lotion but neither helped. She is worried that she can spread it to her children.

On exam, you see a large, purple, polygonal, patch on the flexor surface of her left wrist. When you look closely, you see a white, lace-like pattern on the surface.

What is the most likely cause of this condition?

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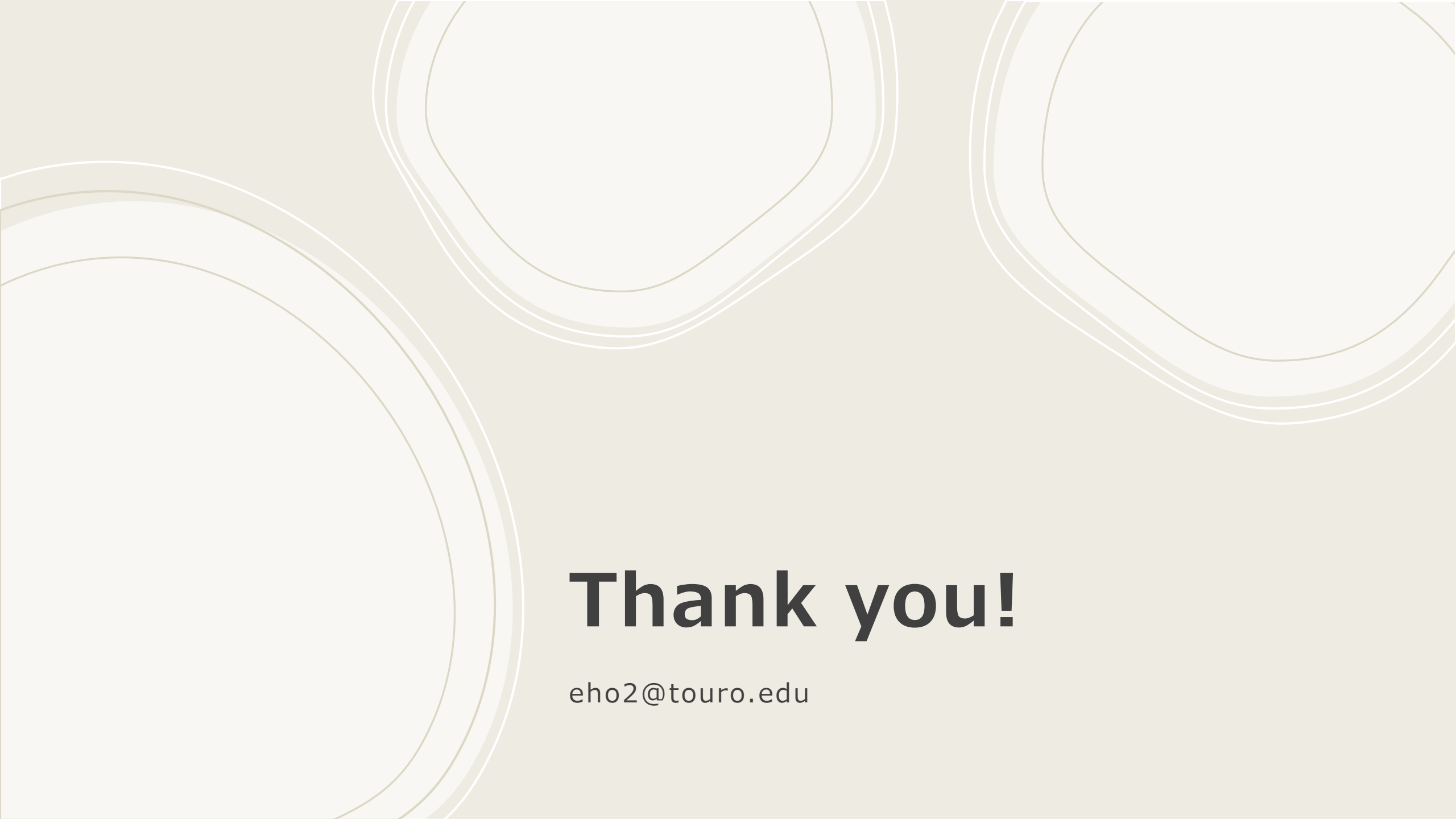
Case 4

A 40 year old female presents to your office with a rash. She states the rash began about 3 weeks ago. It is on her inner wrist and is **extremely itchy**. She tried diphenhydramine and lotion but neither helped. She is worried that she can spread it to her children.

On exam, you see a **large, purple, polygonal, patch** on the **flexor** surface of her left wrist. When you look closely, you see a **white, lace-like pattern** on the surface.

What is the most likely cause of this condition?

- ◆ 1. Psoriasis vulgaris
- ◆ 2. Pityriasis rubra pilaris
- ◆ 3. Pityriasis rosea
- ◆ 4. **Lichen Planus**



Thank you!

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